IN THE COURT OF CRIMINAL APPEALS OF TEXAS

EX PARTE
ROBERT LESLIE ROBERSON III
APPLICANT.

Trial Cause No. 26,162-A
Writ Cause No. WR-63,081-03, 04, 05, 06

EMERGENCY MOTION FOR STAY OF EXECUTION

EXECUTION DATE: OCTOBER 16, 2025

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BASIC FACTUAL AND PROCEDURAL BACKGROUND

I. The Underlying Case

Robert Roberson's daughter Nikki experienced chronic health challenges in her 27 months of life, including a documented history of antibiotic-resistant infections and breathing apnea when she would inexplicably collapse, cease breathing, turn blue, and have to be revived. In the last week of her life, Nikki suffered from a respiratory infection, vomiting, and diarrhea. Her father took her to the emergency room on January 28, 2002. The doctor in the county's only hospital prescribed Phenergan in suppository form.[1] Then Nikki developed a high fever; so, the next morning, her father took her to her pediatrician's office where her temperature was measured at 104.5 degrees. The doctor diagnosed a "respiratory illness, likely viral" —but prescribed more Phenergan, this time combined with codeine. The FDA now restricts administering Phenergan and codeine to children because these drugs can cause breathing difficulties and death.

The next day, January 30, 2002, around 10:00 PM, Robert retrieved Nikki from her maternal grandparents' house at their request. Although the grandparents had agreed to watch Nikki for a few days while Robert's live-in girlfriend was in the hospital, they claimed that Mrs. Bowman (Nikki's step-grandmother) had

[1] Phenergan is the brand name for promethazine.

become ill too. Robert willingly drove out to their house in the country, took Nikki home, got her something to eat, and put her to bed.

In the early morning of January 31, 2002, Robert was awoken by a strange cry and found Nikki on the floor at the foot of the bed, seemingly after falling in her sleep. After comforting Nikki, the two eventually fell back asleep. But when Robert awoke later that morning, he found Nikki unconscious with blue lips.

Robert tried to revive Nikki then rushed to the local ER. But her eyes were already fixed and dilated (a grave sign). Despite efforts to reverse Nikki's condition, she was removed from life support in Dallas on February 1, 2002, after being transferred to the Children's Medical Center of Dallas (CMCD), where a child abuse pediatrician (CAP) diagnosed SBS based on Nikki's triad of internal head conditions: (1) bleeding under the dura membrane outside of the brain (aka subdural hematoma); (2) brain swelling; and (3) retinal hemorrhages. CAP Janet Squires' SBS diagnosis was used to obtain an arrest warrant for Robert moments after Nikki, who had been taken off life support without Robert's knowledge or consent, was pronounced dead. A hasty autopsy was opened and closed the next morning. The same Palestine detective who had arrested Robert the night before sat in on the autopsy after conveying to the medical examiner that Nikki's father had already been arrested for murder.

In 2002-2003, when Robert was accused, tried, and convicted, the medical consensus permitted presuming that a child had sustained inflicted head trauma whenever three internal head conditions were found (subdural hematoma, brain swelling, retinal hemorrhage). The assumption was that, where one or more of these symptoms were present, a child must have been the victim of intentionally inflicted abuse involving shaking and perhaps blunt impact and that whoever had been caring for that child when the symptoms became manifest must have been the culprit because there could be no "lucid interval." Per Dr. Squires and other SBS proponents, absent some verified major trauma, such as a car wreck or a fall from a multistory building, inflicted trauma could be presumed.

Moreover, the medical community then mistakenly believed that short falls, like the one Robert had described, could never cause severe injuries. Based on the presumption that the triad was indicative of abuse, and the belief that short falls could not explain any aspect of Nikki's condition, no other explanation was even considered. Additionally, as CAP Janet Squires noted in making her diagnosis, there did not seem to be any other explanation because Nikki's maternal grandparents had provided Dr. Squires with the incorrect information that Nikki had been "totally well" the night before her collapse.[2] Additionally, CMCD staff

[2] When the Bowmans provided this inaccurate information, they knew she had been to both the ER and her pediatrician days before, that her temperature had been

observed no injuries to the neck, chest, abdomen, genitalia, extremities, or posterior but only one minor abrasion on Nikki's chin and one minor bump on the back of her head with no corresponding fracture of any kind. For Dr. Squires, this absence of external injury was further evidence that Nikki's internal condition could only be explained by shaking.

No one considered Nikki's respiratory illness, investigated her compromised lungs or the respiratory-suppressing medications she had been prescribed, or understood at that time how breathing difficulties can cause bleeding outside of the brain and brain swelling when a disease like pneumonia progresses to the point of sepsis (something that is now recognized by contemporary science).

The SBS hypothesis used to convict Robert has since been entirely discredited by evidence-based science. Today, the medical consensus is that many naturally occurring phenomena, such as Nikki's pneumonia, can cause the triad of intracranial conditions. Today, even those who still believe that shaking, with or without impact, can produce the triad have accepted that a differential diagnosis is required and that abuse can only be a diagnosis of exclusion. That is, today, the medical consensus is that all accidental and natural conditions must be excluded as possible causes of the triad before abuse or inflicted trauma can be posited. In short, support for the SBS measured at 104.5 degrees, and that they had in their possession her prescriptions for Phenergan suppositories, Phenergan and codeine

cough syrup, and Imodium. principles that formed the basis of Robert's prosecution have eroded entirely. Yet the evidence he has amassed of (1) the changes in scientific understanding and (2) other causes of Nikki's medical condition have yet to be considered by any court.

II. Facts Relevant to the Case's Current Posture

Mr. Roberson's case was prosecuted in 2002-2003 by former Anderson County District Attorney Doug Lowe and current Anderson County District Judge Mark Calhoon.[3] The Anderson County trial judge was the Honorable Bascom Bentley.

After Mr. Roberson was convicted, he was appointed counsel for his initial habeas proceeding who, like his appointed trial counsel, refused to investigate his innocence claim (or anything else). The 01 Application contained virtually no cognizable claims and mostly regurgitated what had been raised in the direct appeal (pursued by the appointed trial lawyer who had also ignored his client's claim of innocence). While the 01 Application was pending, Mr. Roberson sent a letter to the Court asking for someone to be appointed to investigate his innocence. That letter was then treated as his "02 Application." There was no evidentiary

[3] On information and belief, current Anderson County District Attorney Allyson Mitchell was employed in former District Attorney Lowe's office when Mr. Roberson was charged in 2002 and tried in 2003. Based on the face of the trial transcripts, however, she did not play a public-facing role in Mr. Roberson's trial.

hearing of any kind. Both the 01 and 02 "Applications" were summarily denied at the same time.

Years passed during which Mr. Roberson continued to send desperate letters to various courts asking for a lawyer who would investigate his innocence.

In 2016, while Mr. Roberson was facing his first execution date, he finally obtained pro bono counsel willing to investigate his claim of innocence who noted the outdated SBS hypothesis that had been used to accuse and convict him. The 03 Application was filed, relying, in part, on Texas's new "junk science writ," Article 11.073 of the Texas Code of Criminal Procedure. This Court stayed his then-pending execution and remanded all four of his claims, including his changed-science and Actual Innocence claims, "to the trial court for resolution." Ex parte Roberson, 2016 WL 3543332 (Tex. Crim. App. June 16, 2016) (unpub.).

Before any further proceedings were held back in the convicting court, Judge Bentley retired. Meanwhile, the presiding judge in the convicting court—the 3rd District Court in Anderson County—was then (and still is) the Honorable Mark Calhoon. But because Judge Calhoon had been one of the two attorneys who had prosecuted Mr. Roberson, he was disqualified from presiding over the writ proceeding. At some point, Judge Evans, then the presiding judge in the 87th District Court, assumed responsibility for the proceeding instead.

An evidentiary hearing in the 03 writ proceeding finally commenced on August 14, 2018, but it was called off that same day after long-lost CT scans taken of Nikki during her final hospitalizations were discovered in a locked closet in the courthouse basement. When these scans of Nikki's head were eventually reviewed by a radiologist (the only kind of medical doctor qualified to do so), that doctor (Julie Mack) found that Nikki had only a single, minor impact site (bump) on the back of her head, not "multiple impact sites" as the medical examiner had claimed at trial.[4] But these exculpatory CT scans have yet to be acknowledged by any court.

The evidentiary hearing resumed on March 8, 2021, at the end of the COVID lockdown. There was a total of eight more days of testimony. Mr. Roberson's counsel presented six experts, establishing the considerable changes in scientific understanding relevant to the testimony put before his jury in 2003, identifying numerous errors and omissions in the autopsy, and outlining evidence that Nikki did not die of an inflicted head injury caused by shaking or otherwise but died instead from natural and accidental causes. None of this evidence has yet been acknowledged by any court.

After the evidentiary hearing record was prepared, the parties submitted proposed Findings of Fact and Conclusions of Law (FFCL). Mr. Roberson's

[4] The medical examiner, Dr. Urban, never reviewed the CT scans or any of Nikki's medical records. Instead, she purported to interpret the intracranial blood itself as evidence of "multiple impacts."

proposed FFCL, summarizing the key evidence in the new 13-volume record, was 302-pages long. The State's proposal, submitted by the Anderson County DA's Office, was 17-pages long and mostly cited the very 2003 trial testimony that had been challenged as outdated and outright false. The State's proposal also denied that the SBS concept had changed much since 2003 and maintained that Nikki had died from inflicted head trauma. The State's proposal did not acknowledge any of Mr.

Roberson's vast new evidence of changes in scientific understanding or of the accidental and natural causes of Nikki's death.**

On February 14, 2022, Judge Evans, who has since recused herself from this case, issued FFCL that largely tracked the State's proposal, including its typographical and grammatical errors. Judge Evans found that SBS is "still an accepted mechanic [sic] of death" and adopted the State's position that Nikki died from inflicted head trauma. Notably, Judge Evans made no finding that Mr. Roberson's experts were not credible; instead, their testimony was simply ignored. Soon thereafter, Judge Evans announced plans to leave the bench.

Thereafter, this Court expressly relied on Judge Evans' misleading FFCL and summarily denied Mr. Roberson relief. Ex parte Roberson, 2023 WL 151908 (Tex. Crim. App. Jan. 11, 2023) (unpub.). At that time, the Court could not have

known that Judge Evans would later (in November 2024) determine that she needed to recuse herself.

The State sought and obtained an execution date of October 17, 2024, from Judge Evans while Mr. Roberson's repeated requests for a hearing were denied.

Soon thereafter, on August 1, 2024, Mr. Roberson filed a new habeas application (the 04 Application). The 04 Application was supported by yet more new evidence of changed scientific understanding and medical opinions. The new evidence expressly rebutted the State's arguments in the 03 proceeding: (1) that the SBS hypothesis had not changed in any material way since 2002-03; (2) that Nikki had not been sick; (3) that the medications she had been given (because she was sick) were irrelevant to understanding her condition; and (4) that the exculpatory CT scans of her head, which the medical examiner had never seen, did not show what the radiologist said they showed.

However, on September 11, 2024, this Court dismissed the 04 Application, without considering the merits. The Court's opinion stated only that Mr. Roberson's five new claims did not satisfy state law procedural requirements. More specifically, the order stated: "We have reviewed the application and find that the allegations do not satisfy the requirements of Article 11.071, Section 5.

See T.CC.P. art. 37.071, § 5(a)."[5]

On October 9, 2024, this Court issued Ex parte Roark, 707 S.W.3d 157 (Tex. Crim. App. 2024) recognizing significant changes in the very same version of SBS (now known as "Abusive Head Trauma" aka "AHT") since the time of Roark's (and Roberson's) trial.

But on October 10, 2024, this Court denied Mr. Roberson's motion to stay his execution and all other pleadings that were still pending. The majority provided no written opinion; however, four judges (the Honorable Newell, Richardson, Walker, and McClure) registered their dissent. Judge Yeary, a dissenter in Ex parte Roark, filed a concurrence, which provided the only substantive explanation as to why at least he believed that relief should be denied. Judge Yeary's concurrence in Mr. Roberson's case, issued the day after this Court's landmark decision in Ex parte Roark, suggested that Mr. Roberson's case was somehow not "just a shaken baby case."[6] Judge Yeary did not explain how his view that Mr. Roberson's conviction is not really a shaken baby case could be squared with Dr. Squires' SBS diagnosis, which was the express basis for the arrest warrant, or why the trial

[5] This statement is confusing because Article 37.071 is a statute that deals only with death-penalty trials, not habeas proceedings—and there is no section 5 in that statute; it only goes to section 2.

[6] A full rebuttal of Judge Yeary's concurrence was submitted to this Court in the 05 Application, which Mr. Roberson has asked the Court to reconsider.

transcripts contain over 200 references to "shaking"—including the medical examiner's views about the mechanics of shaking using SBS jargon.

Additionally, Judge Yeary's concurrence suggests the potential influence of false, extrajudicial reports about what the evidence of Nikki's condition was when she was brought to the ER. Quite simply, there was no evidence of significant trauma to Nikki's head or body when her father brought her to the ER on January 31, 2002, seeking help. As demonstrated with uncontroverted and overwhelming evidence in the 03 proceeding, Nikki had only one minor impact site on her head—a "goose egg," as proven by the CT scans that had been locked up in the courthouse basement for 15 years unbeknownst to Mr. Roberson. The scans show that the single bump on the back of Nikki's head is associated with a small subdural bleed, but no fracture or external bleeding. Dr. Janice Ophoven, a forensic pathologist with decades of experience with pediatric autopsies, demonstrated that the autopsy photographs themselves show that much of Nikki's condition, as observed during the autopsy, was caused by what had happened while she was being treated in the hospital during the two days before the autopsy and even during the autopsy itself:

> you can see discoloration of the skin of her scalp that reflects the blood that has moved there from her ongoing bleeding. This isn't a bruise. This is discoloration from the bleeding that's underneath there. There's

> no impact sites. . . . There are three – four incisions in her skin there [made by the medical examiner], all of which are going to produce bleeding, and one of them – the one that – where the tube [from the pressure monitor] is going into the skin is actually where the tube enters the skull.
>
> So they [hospital staff] had to drill into the bone of the skull, which is going to keep bleeding, and the skin is going to keep bleeding from her problems with clotting. So seeing blood all underneath the scalp skin there, that was done by the doctors. That's not injury.

3EHRR73-74. On October 14, 2024, Mr. Roberson's 05 Application was filed seeking (again) to rely on Article 11.073 and expressly relying on the Court's decision in Ex parte Roark that had issued mere days before. The 05 Application also included a rebuttal to the characterization of the underlying medical evidence in Judge Yeary's concurrence. But on October 16, 2024, the Court again dismissed, without any written opinion, the 05 Application, with four judges again dissenting.

In short, no judicial opinion has yet addressed the vast evidence dismantling the argument that Nikki died from any form of abuse—inflicted via shaking or otherwise. Multiple, highly qualified experts, and even admissions by the medical examiner, show that her 2002 conclusions were contrary to objective evidence of

which she was then unaware or rested on the since-discredited SBS tenets. Because none of that testimony was mentioned in the habeas court's misleading FFCL signed at the conclusion of the 03 proceeding, it is easy to see how members of the Court may have been misled. Thankfully, Mr. Roberson was not executed as intended on October 17, 2024. However, the intense spotlight shining on this case resulted in many false reports about the actual physical evidence in this case circulating through extrajudicial means, seemingly intended to influence how Mr. Roberson and his case would be perceived.

On February 19, 2025, after amassing yet more new evidence, Mr. Roberson filed the pending 06 Application. The 06 Application raises substantial claims of Actual Innocence, changed science, and due process violations based primarily on evidence unavailable when he last filed in October 2024. Among the new evidence supporting the 06 Application is an affidavit from a nationally recognized pathologist, Dr. Michael Laposata, with special expertise in bleeding disorders. As explained further below, Dr. Laposata opined that the light bruising and intracranial bleeding the medical examiner incorrectly labeled as blunt "impact sites" were, in reality, caused by Nikki's DIC, a condition that would have started months before her death from an undiagnosed pneumonia. The 06 Application is also supported by a Joint Statement from ten independent pathologists about vast flaws in the 2002 autopsy.

The 06 Application is still pending. Further, Suggestions to Reconsider on the Court's Own Initiative are pending in the 03-05 proceedings. An amicus brief by the Innocence Project of Texas, an entity very familiar with Mr. Roark's case, has also been filed in support of the Suggestions to Reconsider. See EX1.[7]

Yet, on June 16, 2025, the State filed a motion seeking a new execution date. This motion was filed within days of the Office of Attorney General (OAG) taking over the State's representation, which has been handled since 2016 by the elected DA in Anderson County. The day after the State's motion was filed, on June 17, 2025, an Objection was filed on Mr. Roberson's behalf, noting his pending 06 Application. See T.CC.P. art. 43.141(d). Mr. Roberson's request for a hearing was granted; but on July 16, 2025, Roberson's Objection did not prevail. Newly appointed Judge Austin Reeve Jackson granted the State's motion and set an execution date of October 16, 2025, necessitating this motion. See EX2 (transcript of 07.16.2025 hearing).

GROUNDS FOR A STAY

New grounds argue strongly in favor of staying Robert Roberson's October 16th execution date.

[7] "EX" refers to exhibits to this Emergency Motion.

I. No Law Required Setting the October-16th Execution Date, Which Merely Puts Untoward Pressure on this Court to Rush Adjudication of Pending Claims.

When the State sought the October-16th execution date, its motion all but ignored the fact that Mr. Roberson currently has pleadings pending before a higher court—i.e., this Court. The pending appeals are based on voluminous new evidence only available since this Court last considered his quest for habeas relief. Specifically, Mr. Roberson has a pending subsequent state habeas application pursuant to both Texas Code of Criminal Procedure Article 11.071, section 5(a) and Article 11.073 ("Subsequent Application"). The Subsequent Application is supported, inter alia, by an affidavit from a nationally recognized pathologist with expertise in bleeding disorders, Dr. Michael Laposata. Dr. Laposata has concluded that infection (Nikki's undiagnosed pneumonia) and a serious bleeding disorder made Mr. Roberson's daughter Nikki highly susceptible to internal bleeding and bruising; this condition, not any physical trauma, explains the minor bruising and the internal bleeding observed during Nikki's 2002 autopsy. Mr. Roberson's pending Subsequent Application is also supported by the Joint Statement of ten independent pathologists explaining why the conclusions of the medical examiner who performed the 2002 autopsy are deeply flawed and unreliable. The new evidence in the Subsequent Application supports these new claims:

• Claim 1: Roberson Is Entitled to Habeas Relief Because He Is Actually Innocent;

• Claim 2: Roberson Is Entitled to Habeas Relief under Article 11.073;

• Claim 3: Roberson has Been Deprived of a State-Created Liberty Interest in Violation of Federal Constitutional Law.

Mr. Roberson has also asked this Court to reconsider its previous decisions in his habeas proceedings based on Ex parte Roark, 707 S.W.3d 157 (Tex. Crim. App. 2024). Ex parte Roark was decided mere days before Mr. Roberson's last scheduled execution date. In Ex parte Roark, the Court granted habeas relief under Article 11.073 based on changes in the scientific understanding of the same SBS hypothesis used to convict Mr. Roberson. (And Ex parte Roark involved the exact same SBS expert the State had used in Mr. Roberson's 2003 trial.) Now that the Court has recognized the serious flaws underpinning the SBS hypothesis of that era, there is a compelling reason to revisit the decision to deny Mr. Roberson a new trial based on the habeas court's erroneous FFCL that the SBS hypothesis had not changed since 2002-2003.[8]

Article 43.141 of the Texas Code of Criminal Procedure contains the law relevant to setting execution dates. There is no provision in Texas law requiring a

[8] Ironically, even the name has changed since 2003 to "Abusive Head Trauma," in response to the deluge of criticism arising from evidence-based science.

convicting court to set an execution date at a particular time. Article 43.141 contains only limitations on when an execution date can be set and identifies circumstances when a previously set execution date may be modified or withdrawn:

> The convicting court may modify or withdraw the order of the court setting a date for execution in a death penalty case if the court determines that additional proceedings are necessary on: (1) a subsequent or untimely application for a writ of habeas corpus filed under Article 11.071; or (2) a motion for forensic testing of DNA evidence submitted under Chapter 64. T.CC.P. art. 43.141(d).

Yet the newly assigned judge in the convicting court seems to have believed he had no choice but to set an execution date: "it doesn't seem like anything is going to get resolved without a date to get folks to move, and that's unfortunate, but it's a reality of our system sometimes." EX2 at 18.

Article 43.141(d) expressly grants a convicting court the discretion to withdraw an execution date after a subsequent habeas application has been filed. See also EX3 (samples of orders trial courts have entered withdrawing execution dates). Therefore, basic logic would suggest that it is neither required nor desirable to set an execution date when a subsequent habeas application is already pending.

Indeed, it is highly unusual to seek an execution date under such circumstances. Therefore, during the July-16th hearing, Mr. Roberson's counsel urged the convicting court to decline to set an execution date precisely because of the pending Subsequent Application. See EX2. There was no legal or practical reason to put pressure on this Court to short-circuit the adjudicative process when an execution date could easily be sought if this Court were to deny relief.

Article 11.071, section 5 provides that this Court has sole authority to determine whether to authorize any claim in a subsequent application for further development. Thus, when a subsequent application is pending and no execution date has already been scheduled, a convicting court should not schedule an execution if additional proceedings are necessary. T.CC.P. art. 43.141(d). Since only this Court is authorized to determine whether additional proceedings are necessary on a subsequent application and because Article 43.141(d)(1) expressly provides for withdrawing an execution date after a subsequent habeas application is filed, this Court should grant the instant Emergency Motion for Stay of Execution. Then this Court can continue with the adjudicative process without the ticking timebomb of an imminent execution date.

The convicting court's rationale for setting an execution date at this time was expressed as the need to "get folks to move." EX2 at 18. But there is no statute or caselaw suggesting that putting pressure on a higher court to issue a decision is a

valid basis for setting an execution date. The need for full and fair deliberation should be especially obvious in a case that has been fraught with due process problems and where considerable evidence of innocence has been amassed but not yet considered. See 06 Application.

The State will sustain no harm if its quest for an execution date is temporarily delayed or permanently prevented, as the State has no legitimate interest in executing an innocent person. By contrast, Mr. Roberson will experience irreparable harm if a stay is not granted and the merits of his pending claims are never fully or fairly considered.

Finally, the public interest is not served by rushing this Court's adjudication. The public has no interest in seeing potentially innocent people executed. By contrast, the fair and accurate resolution of this case will serve the public interest.

II. Significant Changes in Scientific Understanding and in New Evidence of Innocence Have Been Adduced Since this Court Last Considered the Merits of Applicant's Claims.

The merits of Mr. Roberson's Actual Innocence and Article 11.073 claims have not been considered since the 03 proceeding, where evidence was closed in March of 2021. In the habeas court's FFCL that issued nearly a year later, there is no mention of most of the evidence that had been amassed during the 03 proceeding—as if Mr. Roberson had not presented six highly qualified experts or

substantial evidence of how the SBS hypothesis, circa 2002-2003, had been completely debunked. There were no adverse credibility findings, only a complete disregard of the habeas record that had been built. But then this Court expressly relied on those misleading FFCL when it denied relief on January 11, 2023, the culmination of serious due process violations. See 06 Application, Claim 3.

Since then, a great deal of additional evidence of Mr. Roberson's innocence and of problems with the SBS hypothesis has been amassed. But no court has yet considered that evidence. All subsequent applications have been dismissed without considering the merits.

Critically, when a habeas applicant brings an Actual Innocence claim in a subsequent application, there is no requirement that he prove that his new evidence of innocence was not "previously available," as is true with other kinds of claims. T.C.C.P. art. 11.071 sec. 5 (a)(1). Texas law requires only that the habeas applicant allege sufficient facts to show that, "by a preponderance of the evidence, but for a violation of the United States Constitution no rational juror could have found the applicant guilty beyond a reasonable doubt." See id. sec. 5 (a)(2). Thus, in deciding the threshold issue of whether Mr. Roberson's pending Actual Innocence claim is procedurally barred, this Court must consider all of the evidence, old and new, and decide whether, under the "easy to meet" preponderance standard, any "rational juror could have found the applicant guilty beyond a reasonable doubt." Id. See

also Ex parte Cobb, 710 S.W.3d 747 (Tex. Crim. App. 2025) (Yeary. J.) (describing the "preponderance of the evidence standard" as "easy to meet"). When this Court applies the appropriate standard to the evidence, old and new, it will conclude that no rational juror could find Mr. Roberson guilty beyond a reasonable doubt since the reasons to doubt his guilt are overwhelming.

Of course, for a habeas applicant to ultimately prevail on an Actual Innocence claim, this Court "must be convinced that new facts 'unquestionably establish' applicant's innocence;" and the term "unquestionably establish" means to establish by "clear and convincing evidence." Ex parte Elizondo, 947 S.W.2d 202, 209 (Tex. Crim. App. 1996). But Article 11.071, section 5 imposes a much lower threshold burden for an Actual Innocence claim to be authorized; and Mr. Roberson has easily satisfied that threshold burden, which also means he has satisfied the "easy" preponderance standard for obtaining relief under Article 11.073.

A. The SBS testimony in the Roark and Roberson cases was virtually identical, a circumstance that has yet to be addressed in any court decision.

This Court granted habeas relief to Andrew Roark following consideration that commenced nearly 11 months after Mr. Roberson had been denied relief in the only habeas proceeding in which his claims were ever considered on the merits—

but where all of his robust evidence of innocence was simply ignored by the habeas judge in flagrant violation of his right to due process.

Mr. Roberson's case involves numerous overlapping issues with those raised in Mr. Roark's case—which Mr. Roberson flagged for the habeas judge before she issued her FFCL, a fact that was also ignored. Additionally, the FFCL did not mention any of the evidence of "changed science" that Mr. Roberson, like Mr. Roark, had introduced into the habeas record.

Both convictions (of Mr. Roark and of Mr. Roberson) hinged on the same version of the SBS hypothesis. Both cases were tried in the same era (2000 and 2003, respectively) when a version of SBS, now universally rejected, was viewed as medical orthodoxy; and both trials featured the very same CAP, Dr. Squires.

In both cases, Dr. Squires opined that three medical findings—(i) subdural bleeding, (ii) cerebral edema aka brain swelling, and (iii) retinal hemorrhage—permitted presuming that abusive shaking, perhaps with blunt impact, had occurred.

The version of SBS put before Mr. Roark's and Mr. Roberson's juries, which permitted presuming inflicted head trauma, has since been rejected—because numerous other phenomena, including accidental short falls with head impact and naturally occurring disease, such as pneumonia, can cause the same

constellation of intracranial symptoms.[9] Likewise, the presumption, embraced by Dr. Squires and other SBS-proponents at that time, that the adult with a child when she became symptomatic had to be the "perpetrator" of inflicted head injury because there could be no "lucid interval," has also been proven false. The cascade of symptoms— subdural bleeding, brain swelling, retinal hemorrhages—can develop over many hours or even days before reaching a crisis point when a child collapses, seemingly for no reason.

Although the Roark case involved a 13-month-old who sustained brain damage but lived, and Mr. Roberson's daughter Nikki was a 27-month-old who died, much of Dr. Squires' trial testimony speculating about the cause of each child's condition is virtually identical—and none of it would withstand scrutiny today.[10] Her testimony described the violent shaking she imagined had occurred, with the child's head flopping back and forth after which the child was, perhaps,

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[9] Mr. Roberson adduced voluminous evidence-based science to support this, including the research of Dr. Patrick Barnes, head of pediatric radiology at Stanford University who had testified for the prosecution in the Louise Woodward "British nanny" SBS case but later established through scientific study that his own previous testimony had been incorrect. See, e.g., Imaging of Nonaccidental Injury and the Mimics: Issues and Controversies in the Era of Evidence-Based Medicine. In this article, Dr. Barnes surveyed a number of conditions, circumstances, and mechanisms that can create the same internal head conditions (subdural blood, brain swelling, and retinal hemorrhages) associated with SBS and thus shows that this triad cannot be used to diagnose trauma, let along inflicted trauma. Id.

[10] Robert was alone with Nikki during the last 11 hours of her life after her maternal grandparents urged him to pick up his sick child around 10:00 PM and take her to his house. Before Nikki's collapse, which was immediately blamed on him, no one had ever reported him harming Nikki—or any other child. Indeed, his

flung down, and left with a massive brain injury. This gruesome, unchallenged hypothetical had to have profoundly affected Mr. Roberson's and Mr. Roark's juries. It was presented to both juries as established fact: that a doctor could look at the child's internal head condition and posit that abuse had occurred without further investigation.

But today, while everyone agrees that violently shaking an infant is horrible, no study has ever shown that shaking can cause any of the internal head conditions associated with the SBS hypothesis (subdural hematoma, brain swelling, retinal hemorrhage). For instance, a study published in 2020 evaluated 36 infants subjected to either admitted or witnessed shaking (with or without blunt force head impact). Of these infants, none who had been purportedly shaken exhibited any element of the triad. See I. Thiblin et al., Medical Findings and Symptoms in Infants Exposed to Witnessed or Admitted Abusive Shaking: A Nationwide Registry Study, 15 PLoS One 8–9 (2020). It is well understood today that shaking, if it causes physical injury, would be experienced first and foremost in a child's neck. (Mr. Roberson's daughter Nikki had no neck injuries of any kind.) extensive school, military, child custody, and TDCJ records do not include any instances of violent behavior. See 7EHRR128-129. Although Dr. Squires and others repeatedly referred to Nikki as a "baby," she was in fact a 27-month-old toddler who weighed nearly 30 pounds; toddlers' necks and brains are markedly different from infants'.

But the identical, now discredited tenets of SBS were applied to the condition of the 13-month-old in Roark's case and to the 27-month-old toddler in Roberson's case as if these two children were anatomically and medically indistinguishable, an approach now seen as unscientific.

Critically, none of the expert testimony put before Mr. Roberson's 2003 jury can withstand scrutiny in light of contemporary scientific understanding. Yet in Mr. Roberson's trial, his appointed counsel approached Dr. Squires with nothing but softballs because, from the outset, he agreed that Nikki's death was a "classic" shaken baby case, as he told the jury, despite Mr. Roberson repeatedly rejecting plea deals and maintaining his innocence. See, e.g., 41RR53-55.[11] In short, Dr. Squires' SBS diagnosis and testimony in Mr. Roberson's and Mr. Roark's cases is no longer viewed as sound science. See, e.g., Keith A. Findley, et al., ed., Shaken Baby Syndrome: Investigating the Abusive Head Trauma Controversy (Cambridge Univ. Press 2023) (the first multidisciplinary treatise documenting the history of the SBS/AHT causation hypothesis and its failure to withstand scientific scrutiny).

[11] By contrast, during Mr. Roark's trial, his retained counsel vigorously cross- examined Dr. Squires and tried to keep her from opining beyond the scope of her expertise. See, e.g., Roark, 5RR12-40.

B. Mr. Roberson has submitted voluminous new evidence that merits careful consideration as the Court decides whether to authorize habeas relief.

By 2016, when Mr. Roberson finally had counsel to raised his changedscience and innocence claims, the invalidity of the SBS hypothesis used to accuse and convict Mr. Roberson had become so obvious that, after his 03 Application was filed, the Anderson County DA's Office and medical examiner Urban tried to distance themselves from SBS, claiming this was not really a shaken baby case but simply a case of "blunt force" head trauma.[12] This argument, however, is belied by a trial record replete with descriptions of "shaking" from both of the State's causation experts (Dr. Janet Squires and medical examiner Dr. Jill Urban). Violent shaking was the way they explained Nikki's intracranial condition; it was also how they explained the absence of anything but minor external injuries (none of which could reasonably be presumed "inflicted"). The State's trial experts also used words like "rotational forces" and "shearing," biomechanical terms commonly used in SBS cases. Yet actual biomechanical engineers have rejected the premise that "shearing" forces generated through shaking can cause any aspect of the triad—because actual scientific study has failed to support that proposition.

[12] Simultaneously, and inconsistently, State's counsel has argued that the "science" associated with SBS has not changed, just the name: "Abusive Head Trauma." This is the position reflected in Judge Evans' FFCL upon which this Court relied in denying Mr. Roberson in the 03 proceeding.

No objective reader of the trial transcripts could fail to see that both the Roark and Roberson cases were "classic" SBS cases in the worst sense: cases presented to the jury as if they were grounded in science when we now know there is no underlying science to support the testimony that the jury heard. Even the most vocal proponents of SBS/AHT, like the American Academy of Pediatrics (AAP), recognize that medical understanding has evolved considerably since 2003—such that there is no legitimate debate that naturally occurring diseases, like pneumonia, and accidental short falls with head impact, can produce the same intracranial symptoms previously (and incorrectly) viewed as "diagnostic" of shaking and abuse.

The Roark case emphasized five principles that are defined as "false testimony," four of which are also part of the false testimony put before Mr. Roberson's jury.[13] Those false tenets are: • Short distance falls cannot cause subdural hematomas. • Shaking alone can cause this injury. • There cannot be a lucid injury after a child is injured. • Retinal bleeding is proof of shaking.

Both Mr. Roark and Mr. Roberson showed how significant problems with the tenets of SBS started to reach the larger medical community, such that the AAP, in 2009, abandoned the SBS name and switched to the undefined term

[13] The fifth category of false testimony addressed in Ex parte Roark was the incorrect belief that rebleeds were not possible, an issue not relevant in this case.

"Abusive Head Trauma" aka "AHT." The name change, however, did not resolve the bigger problem that became increasingly evident over time: no evidence-based research supports the SBS/AHT hypothesis.[14]

To rebut Dr. Squires' trial testimony and show how the relevant science has changed, the Ex parte Roark evidentiary record includes a great deal of expert testimony similar to that which Mr. Roberson presented in his 03 habeas proceeding,[15] including: • an affidavit from forensic pathologist Dr. John Plunkett, who also provided an

affidavit in Mr. Roberson's 03 proceeding about how his profession gradually

began to consider the science-based findings in the field of biomechanical engineering, showing that short falls can cause the triad and be fatal and, by contrast, shaking has not been shown to produce the kind of force necessary to cause subdural bleeding but instead would, in the first instance, injure the neck; • testimony from board-certified pediatrician Dr. John Galaznik, who opined

[14] See generally David Moran, et al.Shaken Baby Syndrome, Abusive Head Trauma, and Actual Innocence: Getting it Right,H.J.HL.&P'12, N.2at209-312(2012).

[15] Mr. Roberson's writ record is far more extensive, including a 13 volume Reporter's Record, with Volume 11 consisting of over 2,240 pages of exhibits.

similarly to board-certified pediatrician and pediatric forensic pathologist Dr.

Janice Ophoven in Mr. Roberson's 03 proceeding, about the change in scientific understanding of SBS even within the AAP and about the circumstances that led to rebranding SBS to AHT; and • an affidavit from biomechanical engineer Chris VanEe analogous to testimony

from biomechanical engineer Ken Monson who testified in Mr. Roberson's 03

proceeding about (1) the lack of scientific evidence that shaking has ever caused a subdural hematoma but would instead injure the neck and (2) the forces that can be generated in a short fall with head impact that can, as opposed to shaking, cause the triad.

But Mr. Roberson went further than Mr. Roark to carry, not just the 11.073 "preponderance" burden, but also the "clear and convincing" actual-innocence burden. He did so by adducing medical evidence from an array of highly qualified experts, explaining what had likely happened to his daughter Nikki—rebutting the conclusion that her death was a homicide. These expert opinions include: • Dr. Janice Ophoven, a licensed M.D. since 1971, board certified in forensic

pathology and anatomic pathology with special training and experience in pediatrics and pediatric pathology: She concluded that Nikki's death should

not have been designated a homicide, in part because there is no scientific basis for looking at an impact site and concluding whether it was intentionally inflicted or the result of an accidental fall. Dr. Ophoven opined that Nikki's internal condition simply meant that she had suffered irreversible damage from oxygen deprivation. Dr. Ophoven explained that anyone who stops breathing and has their heart stop is at risk for the same constellation of internal head conditions. If the brain is deprived of oxygen, brain swelling occurs. Then, as pressure against the brain increases, bleeding into the eyes, which are connected to the brain, can occur. Dr. Ophoven was confident that the precipitating event was not "shaking" or "multiple impacts" to the head. Moreover, she explained that Dr. Urban's autopsy pictures, to which the jury had been subjected, were highly misleading because they did not reflect Nikki's condition when she was brought to the ER but were taken during the autopsy, performed after multiple days of medical intervention had affected Nikki's internal and external condition. 3EHRR13-81; EX45. • Dr. Ken Monson, an expert in biomechanical engineering who directs the Head Injury & Vessel Biomechanics Lab at the University of Utah: He explained the relevant scientific literature and studies showing that the

SBS/AHT assumptions about how shaking would cause internal head injuries

but no neck injuries have been falsified. He also explained how the laws of physics and modeling are utilized to study the injury-impact of falls with head

impacts. Dr. Monson explained how a teddy bear, used as a demonstrative exhibit during Robert's trial, weighing less than a pound, is not a comparable

model in any relevant respect to a 28-pound toddler like Nikki and thus misled

the jury. 5EHRR22-108. • Dr. Carl Wigren, a forensic pathologist who had by then performed over

2,000 autopsies and is a member of the American Academy of Forensic Sciences: He concluded that Nikki's death was not a homicide based on numerous factors, including: (1) the report of a fall off of a bed; (2) the evidence (CT scans and autopsy photographs) showing only a single impact site to the back of Nikki's head that was consistent with the report that she had sustained a short fall; (3) evidence in the toxicology report of dangerous quantities of Phenergan/promethazine now known to suppress the nervous system, in Nikki's bloodstream at the time of autopsy; (4) evidence that,

shortly before her collapse, she had been prescribed Phenergan in two forms and cough syrup with codeine, a narcotic that metabolizes into morphine and further suppresses the nervous system; and (5) evidence that Nikki had undiagnosed pneumonia. Dr. Wigren concluded that these factors had come together to cause an "unfortunate accident," "absolutely not" a homicide, and opined that SBS/AHT played no role in causing Nikki's death. 5EHRR157-244; 6EHRR25; APPX92; APPX95. • Dr. Roland Auer, a neuropathologist board certified in the United States and Canada, who is both a medical doctor and a Ph.D. scientist, the author of a leading neuropathology treatise and over 130 scientific articles in peer-reviewed journals, and a researcher with extensive experience with head trauma, hypoxia, hypoxic ischemia, and pediatric pneumonia: He independently identified factors relevant to assessing the cause of Nikki's death. He concluded that her death could not reasonably be deemed a homicide. As a specialist in brain pathology, Dr. Auer clarified that trauma sufficient to cause internal brain damage would leave external markers on the skin in the form of corresponding bruises/contusions and likely corresponding

skull fractures. He found no evidence suggesting significant trauma to Nikki's head, only one minor impact, "no support for multiple impact sites neither on the brain nor in the skull nor in the scalp," and "no evidence for multiple impact sites whatsoever" but instead found evidence in Nikki's lung tissue of advanced interstitial viral pneumonia. He explained that interstitial viral pneumonia causes hypoxia by disrupting the lung tissue and, if untreated, a cascade of symptoms will result in brain death: oxygen-deprived blood vessels leak into the dura; the blood accumulating outside of the brain causes swelling and increased intracranial pressure; the pressure inside the skull in turn causes retinal hemorrhages. He also noted that the drugs Nikki had been prescribed before her collapse—Phenergan, which depresses respiration, and codeine, an opiate—would have done nothing to address her undiagnosed pneumonia but would have further hindered her ability to breath. 8EHRR55-56. 8EHRR8-144; APPX124; APPX110.

Additionally, in the 03 proceeding, all of the autopsy findings were explained by Nikki's deadly respiratory infection (i.e., pneumonia); the respiratorysuppressing medications she was given because her pneumonia had not

been diagnosed; her apparent short fall out of bed hours before her final collapse; and then the medical intervention she sustained after being resuscitated.

But all of this new evidence was simply disregarded. In fact, the State insisted Nikki did not have pneumonia, that the drugs prescribed to her were all irrelevant to understanding her death, and that there were no problems with the 2003 trial. Yet during that trial, the jury had heard no alterative explanation for Nikki's tragic death. The jury only heard the deeply disturbing SBS hypothesis used to explain disturbing autopsy photos of blood in the subdural space inside his daughter's head. The "shaking" narrative was pounded relentlessly before the jury without any resistance from appointed defense counsel who openly agreed with the State's SBS hypothesis, ignoring his own client.

Importantly, the word "pneumonia" does not appear anywhere in the autopsy report or trial record.[16] Yet during Mr. Roberson's habeas proceeding, counsel for the State inexplicably argued that the newly discovered evidence of pneumonia in Nikki's lung tissue was "nothing new" because pneumonia, the disease, was a thing that existed in 2003. Article 11.073, however, does not require that evidence be adduced of a phenomenon that did not exist for it to be considered a change in scientific understanding from testimony used at trial to obtain a conviction. As

[16] The word "shaking" does not appear in the autopsy report either; but at trial the medical examiner expressly testified that she thought Nikki's blunt force injuries were explained by some unknown combination of shaking and impacts. But she

explained above, no expert questioned the SBS hypothesis at trial. Even Mr. Roberson's counsel overrode their Autistic client's protestations of innocence and conceded that Nikki's death was an SBS case because that was the dominant medical view in 2002-2003. Today, no reputable medical doctor would testify as Dr. Squires and Dr. Urban did.[17]

The habeas judge in Anderson County recommended denying Mr. Roberson relief in 2021 based, in part, on the view that, because some doctors still believe in the discredited SBS hypothesis, the science conveyed to the jury during Mr. Roberson's trial has not changed. That position cannot be squared with the plain language of, or the legislative intent behind, Article 11.073. Nor can that position be squared with the voluminous evidence Mr. Roberson adduced showing how and never looked at the CT scans of Nikki's head showing only a single, minor impact site (bump) on Nikki's head. when the relevant science changed, how testimony seemingly offered by qualified medical experts at trial is indefensible today, and how significant, evidence-based explanations account for his daughter Nikki's collapse. That new evidence completely debunks the notion that Nikki's death was a homicide.

[17] Dr. Urban has admitted that she never considered Nikki's medical history. She also acknowledged that a lot of her cases "run together" (suggesting she had not carefully reviewed her previous testimony in Mr. Roberson's case) and could not identify anything she had learned in the intervening years that would make her doubt her 2002 findings. 9EHRR121, 127.

After his 03 habeas proceeding closed, Mr. Roberson amassed yet more new evidence, correlated opinions from additional medical specialists from multiple disciplines, including a lung pathologist, a medical toxicologist, and a pediatric radiologist: • Dr. Francis Green, an expert in lung pathology with over 46 years of experience: He reviewed Nikki's medical history and her lung tissue under a microscope. His detailed report explains that Nikki's lungs were infected with two different and virulent types of pneumonia—a viral and bacterial infection—which devastated Nikki's lungs, starving her brain of oxygen and causing her death. Dr. Green's detailed analysis supported by images of Nikki's lung tissue, shows that Nikki's pneumonia must have started many days or weeks before her final hospitalization. His report also makes clear that Nikki's lung condition could not have been caused by inflicted trauma nor can it be explained by the fact that she was on a ventilator during her final hospitalization, thoroughly rebutting the State's theory that Mr. Roberson's daughter died from inflicted head trauma. • Dr. Keenan Bora, an expert in medical toxicology and emergency room medicine: He concluded that a post-mortem toxicology report (which Dr.

Urban never considered) shows that Nikki had dangerously high levels of promethazine (brand name Phenergan) in her system, prescribed by two different doctors on two consecutive days. Promethazine is a drug no longer prescribed to children Nikki's age and in her condition because it impairs their ability to breathe and can be fatal. Dr. Bora has concluded that this drug exacerbated Nikki's breathing problems and likely hastened her death from her pneumonia infection, which her body was unable to fight off, leading to sepsis and then septic shock, such that vital organs shut down. • Dr. Julie Mack, an expert in pediatric radiology: She concluded that the initial CT scans of Nikki's head show only a single minor impact site on her head. Dr. Mack reviewed the CT scans discovered in the courthouse basement in 2018, where they had been locked up for 15 years. As interpreted by the only type of expert qualified to read them, the scans corroborate Mr. Roberson's statement at the hospital that Nikki had fallen out of bed and possibly hit her head. Dr. Mack also reviewed chest x-rays of Nikki, some produced to Mr. Roberson's counsel only in 2018 and others only in 2024. Dr. Mack concluded that these chest x-rays corroborate Dr. Green's conclusion

that Nikki died of a fatal lung infection (pneumonia).

Collectively, their reports explain all aspects of Nikki's condition and death, establishing, beyond any doubt, that she died of a severe undiagnosed pneumonia that progressed to the point of sepsis, a demise likely accelerated by the wholly inappropriate medications she was prescribed during her final days that would only have made it harder for her to breathe. See 04 Application, EX5-EX8. No court has yet considered this new evidence because both the 04 Application and the 05 Application based on Ex parte Roark were dismissed on unexplained procedural grounds—expressly without consideration of the merits (and thus without consideration of the new evidence).

Mr. Roberson's pending 06 Application asks the Court to declare Mr. Roberson innocent, grant him a new trial, or at the very least send his case back to the district court for further fact-finding based on yet more new evidence bolstering his already overwhelming case of innocence. That new evidence includes: • New legal and scientific developments that further undermine the invalid SBS

hypothesis used to arrest, convict, and sentence Mr. Roberson to death. These

important developments include Ex Parte Roark, a decision invalidating the wrongful SBS conviction of a man whose case is indistinguishable from Mr.

Roberson's in all material respects; • The affidavit of Dr. Michael Laposata, a pathologist with more than 40 years

of expertise in coagulation and bleeding disorders, explaining that Nikki was experiencing a coagulation disorder at the time of her death, likely caused by her chronic infections, including the undiagnosed pneumonia. That disorder made her very vulnerable to bruising and internal bleeding as medical staff were working to reverse her condition over the course of two days. This new evidence supports the conclusion that Nikki's death was not a homicide; • The joint statement of 10 independent pathologists attesting to the

unreliability of the cause and manner of death conclusions reached by the medical examiner who performed the autopsy on Nikki in 2002 and signed a death certificate all on the same day, without considering numerous highly relevant factors demonstrating her death was not a homicide; • Extensive testimony before the Texas House Committee on Criminal

Jurisprudence in October and December 2024 addressing concerns that this Court has not applied Texas's changed-science law, Article 11.073, as the Legislature intended either generally or specifically in Mr. Roberson's case; • The Interim Report of the Texas House Committee on Criminal Jurisprudence

from November 19, 2024, addressing the correct application of Article 11.073

and the Legislature's serious concerns about Mr. Roberson's wrongful conviction; and • Other new information, including declarations from witnesses attesting to Mr. Roberson's deep love for his daughter and refuting the character smears the State has lodged against him in its relentless efforts to cling to a wrongful conviction.

The new expert report from Dr. Laposata explains that the light bruising and intracranial bleeding the medical examiner labeled as multiple blunt "impacts" were, in reality, caused by Nikki's DIC (disseminated intravascular coagulation), "a major bleeding problem" that causes a person to use up their clotting factors, followed by bleeding throughout the body, a condition that would have stared "months before the events which took her life." As Dr. Laposata's affidavit further explains, "The emergency resuscitation that Nikki underwent in the hospital likely caused bleeding under her skin, producing the bruises that appeared only after her arrival at the hospital. Another sign of Nikki's DIC is the more extensive bleeding observed outside the brain during her autopsy that was not present when CT scans were made of her head soon after her initial arrival at the ER." 06 Application, EX2.

Dr. Laposata's affidavit also explains that he reviewed the reports of three other experts representing a range of medical specialties, explaining exactly how Nikki died, and found their conclusions entirely consistent with his own.

The 06 Application explains that this new evidence, in addition to the wealth of evidence previously presented, but not yet considered, amply demonstrates that no rational juror, if provided with all available information, could have found Mr. Roberson guilty beyond a reasonable doubt.

To date, no court has considered the new evidence showing how Nikki died of pneumonia and the other proof that Mr. Roberson is innocent of any crime.

Critically, no court has yet applied the Court's recognition in Ex parte Roark that the version of the SBS hypothesis used to convict Mr. Roberson is scientifically unfounded. In Ex parte Roark, this Court ruled:

> We find that scientific knowledge has evolved regarding SBS and its application in Applicant's case. Additionally, we find that given further study, the experts would have given a different opinion on several issues at a trial today—some already have. The admissible scientific testimony at trial today would likely yield an acquittal.

Ex Parte Roark, 707 S.W.3d at 185. The 06 Application urges the Court to apply this same legal conclusion to Mr. Roberson's markedly similar case.

To date, at least 41 parents and caregivers in 21 jurisdictions, including Andrew Roark, have been exonerated after being wrongfully convicted under the shaken baby hypothesis, according to the National Registry of Exonerations. As the Parental Rights Foundation[18] noted in urging clemency for Mr. Roberson, many "innocent parents have been set free" as the baselessness of SBS is being recognized. Ex parte Roark seemed to bring Texas in line with legal developments in other jurisdictions that have recognized the baseless nature of the version of SBS/AHT used to obtain convictions circa 2000-2003.[19] And confusion has been sown in the wake of this Court's landmark decision in Ex parte Roark, because of the failure to apply that new precedent to Mr. Roberson's case.

This Court has not yet applied Ex parte Roark to Mr. Roberson's case likely because Ex parte Roark issued mere days before Mr. Roberson's last pleadings were dismissed in October 2024 with an execution date looming. That

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[18] On its website, this entity defines itself as "a research and educational institution whose mission is to educate and inform the public and policymakers regarding parental rights with a goal of 'securing the blessings of liberty to ourselves and our posterity.'" See https://parentalrighttsfoundation.org/.

[19] See, e.g., Jones v. State, 2021 WL 346552 (Md. Ct. Spec. App. Feb. 2, 2021) (granting actual innocence writ after finding version of SBS used to obtained conviction had changed considerably and trial had involved discounting child's medical history); State v. Edmunds, 746 N.W.2d 590, 595, 598-99 (Wis. Ct. App. 2008) (finding new evidence related to SBS/AHT controversy "[was] entirely different in character from the evidence offered" at trial as it showed "a shift in mainstream medical opinion" and granting new trial); Commonwealth v. Epps, 53 N.E.3d 1247, 1268 (Mass. 2016) (finding "substantial risk of a miscarriage of justice where the jury heard no scientific or medical expert challenging the majority views on [SBS/AHT] and short falls, and where new research has emerged since the time

"unfortunate" circumstance has now been reproduced—imposing undue and needless pressure on the deliberative process. EX2 at 17 (Judge Jackson characterizing his own decision to set a new execution date as "unfortunate").

The Roark and Roberson cases involve essentially the same discredited science challenged using the same Texas law: Article 11.073. In both cases, the State even relied on the same CAP (Dr. Squires) whose same discredited SBS diagnosis of trial that would lend credibility to the opinion of such an expert" and granting new trial); People v. Bailey, 41 N.Y.S.3d 625, 627 (App. Div. 2016) (holding that "advancements in science and/or medicine may constitute newly discovered evidence" and "defendant established . . . that 'a significant and legitimate debate in the medical community has developed in the past ten years over whether infants [and toddlers] can be fatally injured through shaking alone'" and affirming lower court ruling vacating SBS/AHT conviction) (alteration in original, citation omitted); Jones v. State, No. 0087, 2021 WL 346552, *11-20 (Md. Ct. Spec. App. Feb. 2, 2021) (surveying changes in SBS/AHT's original tenets in light of scientific developments and finding that petition for writ of actual innocence should be granted); Kaiser v. State, ___ N.W.3d ___, 2024 WL 1080968 (Minn. March 13, 2024) (finding that SBS/AHT testimony from State's expert witnesses was false testimony that child's medical condition was the result of intentionally inflicted head trauma); see also Del Prete v. Thompson, 10 F. Supp.

3d 907, 951 (N.D. Ill. 2014) (finding petitioner convicted for 2002 death of child in SBS case had satisfied Schlup v. Delo, 513 U.S. 298 (1995) gateway materiality standard for constitutional claims). was used to accuse and convict two men who happened to be caring for a medically fragile child when she experienced a medical emergency.

Recognizing that scientific understanding has changed should not hinge on the whims of one DA's Office or whether a particular medical examiner is willing to revisit decades-old opinions and look at overwhelming new evidence from far more experienced and qualified medical experts. Indeed, it is the epitome of arbitrariness when individual medical examiners in some, but not all, jurisdictions can admit that their opinions in SBS cases have not withstood the test of time and when many, but not all, courts can do the hard work of reviewing new evidence and acknowledging significant changes in the scientific understanding of SBS.[20]

A stay will allow this Court to assure that the legislative intent underlying Article 11.073 is being applied consistently throughout Texas. But if Mr.

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[20] See, e.g., State v. Hunter, No. B 0600596 (Ohio Com. Pl. May 10, 2023) (granting habeas relief after medical examiner, who had relied on SBS/AHT hypothesis at trial, recognized new evidence that child's injuries were consistent with a short fall and that the anal and rectal wounds, previously used to support a sexual assault conviction, had been medically inflicted by hospital staff); People v. Liebich, No. 2-13-0894, 2016 WL 1222198 (Ill. App. Ct. Mar. 28, 2016) (2002 SBS conviction vacated where medical examiner admitted that medical records revealed a condition that could have started two-year-old's decline days before child's collapse); Johnson v. Felker, No. CV-07-357-RHW, 2010 WL 1904858 (E.D. Cal. May 10, 2010) (2002 SBS conviction vacated after medical examiner acknowledged that, in light of new scientific evidence, child's head injuries could have been caused by accidental fall).

Roberson's execution proceeds before that can be accomplished, an unspeakable miscarriage of justice will ensue. Granting this Emergency Motion for Stay of Execution, so that this Court can continue to assess Mr. Roberson's significant claims, is paramount. Fair assessment of his claims is not just a matter of individual justice but relevant to the integrity of the state's criminal justice system.

CONCLUSION & PRAYER

For the foregoing reasons, given the urgency and complexity of his case and all relevant law and equities, Robert Roberson respectfully prays that this Court STAY his October 16, 2025, execution as the Court considers the merits of the claims in the 06 Application and considers taking the initiative to reconsider the denial of relief under Articles 11.071 and 11.073 in the 03 proceeding and/or the dismissal of the claims raised in his 04 and 05 Applications without merits consideration. Mr. Roberson also prays for any further relief that justice requires. Respectfully submitted, /s/ Gretchen S. Sween Gretchen S. Sween Texas Bar No. 24041996 712 Upson Street Austin, Texas 78703 214.557.5779 (telephone) 512.548.2089 (fax) gsweenlaw@gmail.com Callie Heller Texas Bar No. 24101897 Western District of Oklahoma Federal Defender Office Capital Habeas Unit 215 Dean A. McGee, Suite 707 Oklahoma City, OK 73102 (405) 609-5975 (405) 609-5932 fax Callie.Heller@fd.org

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